Clinical Immersion Clerkship Syllabus
Part 1: Course Information

Course Name: Surgery

Course Number: 3SRG01
Academic Year: 2026-2027

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Course Description

During this 8-week rotation students will be introduced to the management of surgical diseases – including anatomy, pathophysiology, diagnosis and treatment. They will gain practical clinical experience in operating rooms, the emergency department, intensive care units, floors and clinics. These settings will expose them to faculty, residents, other care providers, as well as patients and their families. There are detailed objectives for the Clerkship. During the rotation, students are expected to encounter specific conditions, and perform or participate in specific procedures. In both cases these may be real or simulated encounters. Practical clinical experience will be supplemented by appropriate educational experiences in various conferences, as well as engagement in self-directed study to prepare for patient encounters, an oral exam, and the Shelf exam, which is taken at the end of the rotation.

During the rotation students will be exposed to a variety of general surgery and specialty surgical services. All students will have exposure to emergency surgery, trauma surgery, specialty service rotations, general surgery and overnight shifts. While students cannot experience every surgical service, below are the services available to students.

Services that students may rotate on include: Emergency General Surgery, Trauma Surgery, Burn Service, Surgical Oncology services, Transplant Service, Community General Surgery, Pediatric Surgery, Hepatobiliary Surgery, Minimally Invasive Surgery, Colorectal Surgery Service, Ophthalmology, Vascular Surgery, Cardiothoracic Surgery, ENT, Plastic Surgery, Orthopedics, Neurosurgery, and Urology.

Students are expected to become an instrumental member of the team in each service, undertaking tasks that advance patient care, as directed by faculty, fellows or residents. Examples of these tasks are pre-rounding on patients, taking histories and performing physical exams, presenting patients to more senior members of the team, and assisting in the operating room.

Students' performance is assessed for each of 5 components of the rotation, i.e. clinical skills, clinical documentation, Shelf exam, professionalism, and completion of administrative tasks. Final grades of Honors and High Pass require high standards of performance in each of these components.

Credits, Hours, and Prerequisites

Length of Course:	8 weeks
Clerkship Environment:	Inpatient/Outpatient/Both
Prerequisite(s):	Successful completion of second year
*Benchmark/Gateway Course:	Yes

**Benchmark/Gateway courses affect the ability of a student to advance through the medical school curriculum.*

Methods of Instruction

Clinical settings

Much of students' learning will take place in clinical settings – clinics, floors, the emergency department, intensive care unit, and operating rooms.

Conferences

Students should attend any conferences (unless specifically excused attendance) run by the services on which they rotate. Services generally have at least one such conference per week. Information about these conferences will be available from the service coordinator, or faculty / residents on the service.

While on a general surgery service, students should attend the core conference (QPI and Grand Rounds), unless specifically excused. In Winston-Salem, conferences occur on Wednesday mornings between 6:30 am and 8:30 am. In Charlotte, conferences occur on Thursday mornings between 7 am and 8:30 am.

A faculty facilitated didactic specifically for students occurs approximately weekly. Students will be notified by the coordinator of the time, location, topic, and appropriate preparation for these conferences. Students should ensure that they are sufficiently informed to engage in a meaningful discussion on the conference topic. They should also notify their team in good time that they are required to attend these conferences. They are excused from attendance if doing so would result in duty hour violations (this is likely to occur only while on night service). The Clerkship Director must be notified of any such absence.

Students should be aware that as in clinical settings, patient-related material discussed at these conferences is confidential.

Self-directed study

In order to prepare for patient encounters, the Shelf exam, as well as to continue to develop life-long learning skills, students are expected to identify their own learning needs and ensure that they consult appropriate resources to meet those needs. Some such resources are listed below in Recommended Resources.

Textbook, Course Materials, and Resources

- **Required Resource**: UWorld QBank
- **Recommended Resources:**
  - Essentials of general surgery, 6th edition, [edited by] Peter F. Lawrence
  - American College of Surgeons, ASE Medical Student Core Curriculum
    - https://learning.facs.org/content/acs-ase-medical-student-core-curriculum
  - Surgery: A Case Based Clinical Review (Christian de Virgilio)
  - Kaplan Surgery Notes
  - NMS Surgery CaseBook
  - Surgical Recall (Lorne H. Blackbourne)
  - Aquifer / Wise-MD
  - OnlineMedEd
  - Subspecialties may recommend additional texts.

Technology Requirements

- Internet connection
- Access to Canvas, the learning management system used by Wake Forest University School of Medicine and other required online content
- Access to WFUSM.edu email

Part 2: Course Details

Clerkship Success

If you find that you have any trouble keeping up with assignments or other aspects of the clerkship, make sure you let your clerkship director and coordinator know as early as possible. As you will find, building rapport and effective relationships are key to becoming an effective professional. Make sure that you are proactive in informing your clerkship director and coordinator when difficulties arise during the clerkship so that we can help you find a solution.

Grade Assignment (Grading Scale)

The assignment of grades is based on levels of achievement earned in each of the components below: clinical skills, clinical knowledge, and professionalism. A student must meet the minimum passing threshold of each component to pass. The final grade is determined by the Clerkship Grading Committee. The evaluation committee cannot make exceptions to the thresholds for each component because they are the UMECC approved standardized thresholds across all clerkships.

The final grade will be posted once all required elements of the clerkship have been completed and/or received. A grade of I (incomplete) will be posted if all clerkship requirements are not complete prior to the end of the clerkship. Students must complete a Remediation/Course Completion Agreement with the Clerkship Director if an incomplete is posted. The final grade will be posted once all required elements of the clerkship have been completed in accordance with the Remediation/Course Completion Agreement. Please refer to the SAP policy in Policy Tech for further details.

Final Grade	Points
Honors	90 - 100
High Pass	80 - 89.9
Pass	60 - 79.9
Fail	<60

How the Points/Score is Calculated	
Graded Components	***Weight***
Clinical Skills	***60%***
Clinical Evaluation Form Q1-6 Minimum Passing∞ Threshold ≥ *60%*	60%
Clinical Knowledge	***30%***
NBME Subject Matter Exam* Minimum Passing∞ Threshold by Equated Percent Correct (EPC) ≥ 61	30% ^
Professionalism ϕ	***10%***
Clinical Evaluation Form Q7-9 Minimum Passing∞ Threshold ≥ *60%*	5%
Administrative Points See narrative below**	5%

Φ The Professionalism component is comprised of the Y3 Clinical Evaluation form Questions 7-9, **and** the final number of Administrative Task points, **and** whether there is a specific professionalism concern(s). Specific comments are reviewed carefully, and additional information is sought from all parties involved if there is a concern. Professionalism and/clinical skills concerns may not be immediately reflected in an evaluation form but may have been brought to the attention of the student, resident, attending physician, and/or Clerkship Director. Significant professionalism concerns can lead to a lower or failing grade for this clerkship, independent of performance on any other aspect of the clerkship. Examples of significant professionalism concerns can include, but are not limited to, concerns with interactions with patients, healthcare team (faculty, house staff, staff, and peers), course staff (coordinators, instructors, administrative staff), and/or course directors; professional development; lack of or late responsibility fulfillment and task completion by stated deadlines; a low or concerning level of engagement and/or work ethic; receptiveness to feedback as perceived by the attending, resident or clerkship director; repeated tardiness; failure to reply to coordinator, faculty or course director communications in a timely manner which includes email, direct conversations, text messages, and/or secure chat; and failure or refusal to meet for required or requested feedback session(s) or requested meeting(s) including but not limited to mid-rotation and/or end of clerkship meetings. The clerkship grading committee, clerkship director, and/or Associate Dean of Clinical Education, has the authority to define a concern as being "significant." The clerkship grading committee and/or the clerkship director will determine a specific professionalism concern's effect on a student's final grade for this component.

* Per the institutional grading policy, the highest final grade that can be earned is a Pass if a student does not pass the NBME subject matter exam on the initial attempt.

** Each student is given 100 points at the beginning of the clerkship related to both administrative tasks and professionalism. Each instance of a missed task or item results in losing 10 points. Students who end the clerkship with a final Admin Task score of 70 or less may be referred for support in developing their professionalism skill set. A final Admin Task score of 60 or less, and/or continuing to require repeated reminders, and/or not completing required clerkship assignments/elements, can meet criteria as a specific significant professionalism concern. A final Admin Task score of 60 or less will result in a Final Grade no higher than Pass. See Policy Tech for Student Handbook & SPARC Policy regarding professionalism expectations. Examples of point deductions include, but are not limited to: an assignment or assigned task not submitted by the stated deadline, failure to notify the clerkship director/coordinator when absent from a scheduled educational activity due to urgent direct patient care responsibilities or personal illness / emergency, repeated tardiness to clinical duties or scheduled educational activities, repeated failure to respond to communication from the clerkship director or coordinator within a reasonable time frame considering day/night clinical schedule, and failure to send evaluations to faculty and residents with whom the student has worked. Notification of Point Deductions: Admin points deducted prior to the required Mid-Rotation Formative Feedback meeting will be discussed at that meeting. For admin points deducted after the mid-rotation meeting, the student will be notified by the Clerkship Coordinator or Clerkship Director.

^ To determine the clinical knowledge component weight to calculate the final grade, use the graph below to convert the NBME subject matter exam score percentile to assigned points

Shelf Percentile Range	**Assigned Points**
90 – 100th percentile	100 points
80 – 89th percentile	95 points
70 – 79th percentile	90 points
60 – 69th percentile	85 points
50 – 59th percentile	80 points
40 – 49th percentile	75 points
30 – 39th percentile	70 points
20 – 29th percentile	65 points
7 – 19th percentile	60 points
0 – 6th percentile	Fail

∞ In order to pass this clerkship, students must demonstrate a minimum passing threshold score for each of the following components: Clinical skills, Clinical knowledge, and Professionalism.

Evaluation

Formative assessment / feedback

Students should receive feedback from faculty, fellows and residents with whom they work. Students are strongly encouraged to ask for this feedback on a regular basis.

Mid-Rotation Feedback

Students are encouraged to seek formative feedback throughout the rotation. Students will also need to schedule time for mid-rotation feedback with the clerkship director and/or their attending physician, per the clerkship's direction. If, at the time of mid-rotation feedback, there are concerns regarding student performance in clinical skills, medical knowledge base, and/or professionalism, the course director will provide feedback and work with the student to develop strategies for improvement. If there are concerns regarding professionalism, the Clerkship Director will work with the Associate Dean of Clinical Education to determine if a formal professionalism concern report will be submitted. If concerns are noted after mid-rotation feedback, persist throughout the clerkship, or are reported/discovered after the clerkship has concluded but before the grade is posted, the clerkship director will seek additional information from faculty, residents, other students, and/or staff, and may request a subsequent meeting with the student to discuss these concerns. The final grade will accurately reflect all concerns.

Each student will meet with the clerkship director for a mid-rotation feedback session approximately 4 weeks into the rotation. The self-assessment, part of the Common Clerkship Mid-Rotation Feedback Form (available from Canvas), should be completed, and the form taken to the meeting with the clerkship director. **It is required to obtain feedback with a faculty member with whom there has been substantial contact prior to meeting with the clerkship director. The faculty member should fill out the faculty portion of the mid-rotation feedback form.** After the meeting, the form must be handed in to the Course Coordinator.

Summative assessment / grades

The final grade for the rotation is derived from a number of components, as listed below.

Professionalism Score

Professionalism is assessed by the preceptors named in Acuity One45. Their assessment will be based on their observation of your performance in the domains listed below and compared with expectations for your level of training using the common Y3 Immersion Evaluation form. The Professionalism Score is calculated using the last three questions on the evaluation form.

Clinical Score

Clinical performance is assessed by the preceptors named in Acuity One45. Their assessment will be based on their observation of your performance in the domains listed below and compared with expectations for your level of training using the common Y3 Immersion Evaluation form. The Clinical Score is calculated using the first six questions on the evaluation form.

Shelf exam

On or about the last day of the rotation, you will sit the NBME Surgery shelf exam. This is a web-based, 2½ hour 100 multiple choice question exam. Medical Education or the Clerkship Coordinator will provide additional details about the exam closer to that time. You should receive your results within a week. The topics covered by the exam, and their relative weightings, are reproduced from the NBME page for Surgery. Resources that may prepare you for this exam (as well as for clinical encounters) are listed below under "Suggested Text & Web-Based Resources". These include online resources from Aquifer / WISE-MD and OnlineMedEd

Clinical documentation

1. You are expected to write at least one inpatient note per day. Notes should abide by the Institution's lists of approved & prohibited abbreviations posted on Canvas. Notes will be an *H&P/Consult, a Discharge Summary or a Daily Progress Note*.
2. Feedback sessions regarding your documentation are strongly encouraged. These can be done with a PGY 2 or higher. Please have the feedback sessions signed off on your documentation card. Please turn in your documentation card by the 5th week to your campus' clerkship coordinator.

Professionalism and Administrative Tasks

All students are expected to carry themselves in a professional manner in all activities pertaining to the course. Professionalism is evaluated both globally and in the Y3 clinical evaluation form. It is expected that students will be on time and prepared for all clerkship activities.

All assignments (listed in Canvas and/or discussed at orientation or relayed via email) must be turned in by the due date specified by the clerkship director. Repeatedly late assignments or the need for repeated reminders may result in a discussion with the course director and may result in a decrease in the professionalism grade. Please see details in the grading rubric above.

Students are responsible for submitting the names of the APPs, interns, residents, fellows, and faculty they work with on each rotation so that students can receive evaluations from those individuals.

Professionalism concerns will result in discussion with the course director as well as any steps as defined in the Professionalism Concern Reporting SOP.

Students are expected to complete all campus specific assigned administrative tasks in a timely fashion. These tasks include:

- Completion and return of the mid-rotation feedback forms.
- Completion and return of the duty hour log at the end of your rotation, together with immediate notice to Clerkship Director of any duty hour violations, **including those where the student has voluntarily exceeded hours for educational purposes.**
- Completion and return to the Clerkship Coordinator, by the midpoint of the rotation, of the H&P, mid-rotation feedback, and documentation feedback card provided at orientation

Final grades are determined by the clerkship evaluation committee, which includes the Clerkship Directors. The previously shown table indicates the standard required for each component of the grade.

SHELF Exam Information

If a student does not pass the shelf on the first attempt, the student will be required to retake the shelf and obtain a passing score. Per the institutional grading policy, the highest final grade that can be earned is a Pass if a student does not pass the shelf on the initial attempt. Upon passing the repeat attempt, the shelf component of the final grade will be listed as a Pass.

SHELF Exams are typically scheduled on the last day of the clerkship. If you miss a SHELF exam due to an illness or other extenuating circumstance, your make up exam should be scheduled on a posted exam retake date on which you do not already have a clinical assignment.

Students must take the clerkship shelf exam at the scheduled time with their block cohort if they have completed at 50% or more of the clerkship. If less than 50% of the clerkship was completed because of an approved absence (as outlined in the SSAS rules), the shelf exam will be taken at the next scheduled approved availability, or no later than during LEAP week, remediation week, or vacation week, whichever is approved and occurs earliest.

Clerkships Specifics:
At or before the start of the rotation you will receive a voucher enabling you to take the NBME Clinical Science Mastery Series exam for surgery. You may decide when to sit this examination, provided that you do so before the Friday before the full Shelf exam. The results you receive from the NBME should provide you with information on where you do well, and on what topics you might want to concentrate your studies. They are formative only – they are not a factor in your grade. **Note that even if your results on this test indicate that you have areas of weakness, no additional study time can be made available to you; nor can your full Shelf exam be delayed.**

Shelf exam topic areas: https://www.nbme.org/subject-exams/clinical-science/surgery

General Principles	
Organ Systems	
Immunologic Disorders	1%–5%
Blood and Lymphoreticular System	5%–10%
Nervous System and Special Senses	5%–10%
Cardiovascular Disorders	10%–15%
Respiratory System	8%–12%
Gastrointestinal System	20%–25%
Male Reproductive System	1%–5%
Renal and Urinary System	3%–7%
Female Reproductive System and Breast	3%–7%
Skin and Subcutaneous Tissues	1%–5%
Musculoskeletal System and Connective Tissue	3%–7%
Endocrine Disorders	3%–7%
Multisystem Process and Disorders	5%-10%
Social Sciences (Medical ethics, palliative care)	1%-5%
Physician Tasks	
Applying Foundational Science Concepts	8%–12%
Diagnosis: Knowledge Pertaining to History, Exam, Diagnostic Studies, and Patient Outcomes	50%–60%
Pharmacotherapy, Intervention and Management	30%–35%
Site of Care	
Ambulatory	35%-40%
Emergency Department	25%-35%
Inpatient	30%-35%
Patient Age	
Birth to 17	8%-12%
18-65	60%-70%
65 and older	20%-25%

Graded Clerkship Activities

All lectures, skills labs, and group discussions are required. Any unexcused absence will be reflected in the final professionalism grade. Other than the stated clinical and professionalism grades, all other activities are pass/fail based on participation and attendance.

Required Clinical Skills and Conditions

Chief Concern, Skill, or Procedure	Patient Type/Age	Student Role *(Participate/Perform)*	Goal *(Number of events)*	Setting	Alternative
Chief Concern					
Abdominal Pain		Observe	1	Inpatient or Outpatient	Aquifer/Wise-MD- Appendicitis, Cholecystitis, Pancreatitis (choose one)
Neoplastic Process		Observe	1	Inpatient or Outpatient	Aquifer/Wise-MD- Breast Cancer, Colon Cancer, Lung Cancer, Skin Cancer (choose one)
Abdominal Wall Mass or Hernia		Observe	1	Inpatient or Outpatient	Aquifer/Wise-MD- Inguinal Hernia, Pediatric Hernia (choose one)
Bowel Obstruction or Ileus		Observe	1	Inpatient or Outpatient	Aquifer/Wise-MD- Bowel Obstruction
Diseases of the Skin and Soft Tissue		Observe	1	Inpatient or Outpatient	Aquifer/Wise-MD- Burn Management, Skin Cancer (choose one)
Management of the Traumatically Injured Patient (Blunt, Penetrating, Burn)		Observe	1	Inpatient or Outpatient	Aquifer/Wise-MD- Trauma Resuscitation
Vascular Disease or Vascular Injury		Observe	1	Inpatient or Outpatient	Aquifer/Wise-MD- Abdominal Aortic Aneurysm, Carotid Stenosis (choose one)
Procedures/Skills					
Laceration / skin closure (Laceration repair)		Perform	2	Inpatient or Outpatient	Suture Clinic
Removal skin sutures or staples		Perform	1	Inpatient or Outpatient	Suture Clinic
History & Physical		Perform	1	Inpatient or Outpatient	NA

Clerkship Educational Events

The table below describes the clerkship activities including activities, date, and location in the course. The first column describes the activities that are to be completed. The second column describes the due date/ when in the course the activity is conducted. The third column describes the location where the activity takes place (in person, on Canvas, via Webex, etc).

Clerkship Activity	Due Date/ When Conducted	Location
Required Didactics		
	Upper GI Lecture	Various locations
	Lower GI Lecture	Various locations
	Hepatic, Pancreas, Biliary Lecture	Various locations
	Skin and Soft Tissue Lecture	Various locations
	Trauma Lecture	Various locations
	Diseases of Thoracic cavity (thoracic and foregut) Lecture	Various locations

	Disease of endocrine system (breast, thyroid, parathyroid)	Various locations
	Orientation	Various locations
	Suture Clinic/Workshop	Various locations
Optional Didactics		
Rounds/Conferences		
	Quality Improvement- weekly	Various locations- service dependent
	Grand Rounds- weekly	Various locations- service dependent

Other Important Clerkship Information:

All relevant information to the clerkship is provided in the syllabus

Online resources – Aquifer/Wise-MD

- To access these modules, Register then:
- Log in
- Click "Launch Wise-MD"
- Select modules to review

Disease Process	Duration (minutes)	Skill	Duration (minutes)
Abdominal Aortic Aneurysm	44	Best Practices	39
Adrenal Adenoma	28	Epidural Placement Technique	8
Anorectal Disease	31	Foley Catheter Placement	13
Appendicitis	33	Suturing and Instrument Tie	7
Bariatric	15	Two Handed Knot Tie	4
Bowel Obstruction	46	Ultrasound Basic Principles	5
Breast Cancer	44	Ultrasound: ABI	2
Burn Management	13	Ultrasound: Abdominal Aortic Aneurysm	4
Carotid Stenosis	48	Ultrasound: Breast	3
Cholecystitis	33	Ultrasound: Carotid Artery	4
Colon Cancer	43	Ultrasound: Cholelithiasis / Cholecystitis	5
Diverticulitis	23	Ultrasound: E-Fast Exam	5
Hypercalcemia	20	Ultrasound: For Vascular Access	3
Inguinal Hernia	23	Ultrasound: Thyroid	2
Lung Cancer	30	Ultrasound: Venous	2
Pancreatitis	19		
Pediatric Hernia	16		
Pediatric Pyloric Stenosis	14		
Skin Cancer	26		
Thyroid Nodule	45		
Trauma Resuscitation	25		

Online resources – OnlineMedEd

Surgery: General	Surgery: Subspecialty	Surgery:Trauma
PreOp Eval	Peds: First Day	Shock
PostOp Fever	Peds Weeks to Months	Head
Other PostOp	Endocrine Diseases	Neck
Obst Jaundice	Surgical Hypertension	Chest
Esophagus	Peds CT Surgery	Abdomen
Small Bowel	CT Surgery	Burns

Pancreas	Vascular	Bites and Stings
Gall Bladder	Peds Ophtho	
Colorectal	Adult Ophtho	
Abdm Pain	Skin Cancer	
Leg Ulcers	Neuro Brain Bleeds	
Breast Cancer	Uro Peds	
	Uro Cancer	
	Uro Misc	
	Ortho Hand	
	Ortho Peds	

Part 3: Objectives and Goals

Clinical Immersion Goals and Objectives

This year-long clinical experience reflects the principles of the Wake Forest MD Program in that it reflects our commitment to:

- Empathy and respect for patients and colleagues
- Ethical decision making
- Health equity
- Intellectual rigor and scientific curiosity
- Leadership and innovation
- Patient-centered, team-based care
- Self-inquiry and lifelong learning
- The belief that even the most challenging problems can be solved

The experience will be foundational for all medical students and will serve as an introduction to multiple fields of medicine. Students will have the opportunity to learn and practice skills that will help prepare them for successful residencies and careers in any field. While there are a variety of demands on students completing the Clinical Immersion Year, including time demands, self-study and exam preparation, and choosing a career/medical discipline, we believe that high quality, safe, coordinated patient care is the primary concern. In addition, a holistic educational experience, regardless of planned career, should be another focus for all students.

By the completion of the Clinical Immersion Year, our students are expected to:

1. **Knowledge for Practice** –demonstrate knowledge of the basic, clinical, and behavioral sciences, as well as the ability to apply that knowledge to patient care. Students are expected to:
    a. Maintain sufficient clinical knowledge that can be translated to patient care
    b. Routinely identify the need to seek new information in the context of clinical care
    c. Complete all required course work for clinical rotations, including clinical responsibilities, assignments, and administrative tasks
    d. Attain a passing grade for required NBME Subject Exams

2. **Interpersonal and Communication Skills** –communicate and interact effectively with patients, their families, and members of the inter-professional healthcare team. Students are expected to:
    a. Demonstrate empathy
    b. Engage in patient-centered interviewing and communication
    c. Provide an oral presentation of a clinical encounter that is well organized and accurate
    d. Document a clinical encounter in the patient record
    e. Accurately document the reasoning supporting the clinical decision making

3. **Patient Care** –function as a member of an interprofessional healthcare team and provide patient care that is compassionate, appropriate, and effective for the treatment of health problems and the promotion of health in diverse populations and settings. Students are expected to:
    a. Gather a history and perform a physical exam
    b. Demonstrate clinical reasoning by gathering information relevant to a patient's care through history gathering, physical exam performance, and chart review
    c. Develop and prioritize a differential diagnosis following a clinical encounter
    d. Explain the clinical reasoning that led to the differential diagnosis, working diagnosis, and management plan
    e. Recommend and interpret common diagnostic and screening tests
    f. Describe the implications and urgency of an abnormal result

g. Recognize patients with life-threatening and emergent presentations, and engage in appropriate management (triage, initial therapy, etc.)

4. **Professionalism** –demonstrate a commitment to upholding their professional duties, guided by ethical principles. Students are expected to:

a. Maintain a professional image in attire, grooming, and behavior
b. Display compassion, empathy, and respect for all patients, regardless of background, gender identity, race, ethnicity, and/or sexual orientation
c. Be punctual and appropriately prepared for clinical responsibilities
d. Establish and maintain rapport with patients and families
e. Demonstrate honesty and integrity in all interactions, including with patients, patients' families and support systems, colleagues, and other health care professionals

5. **Practice-Based Learning and Improvement** –investigate and evaluate their care of patients, to appraise and assimilate scientific evidence, and to continuously improve patient care based on constant self-evaluation and life-long learning. Students are expected to:

a. Demonstrate the ability to set learning and improvement goals
b. Demonstrate an understanding of the need to engage in lifelong learning

6. **Systems-Based Practice** –demonstrate awareness and understanding of the broader health care delivery system and strive to use system resources to effectively provide patient-centered care that is compassionate, appropriate, and safe. Students are expected to:

a. Demonstrate responsibility for their actions and errors
b. Recognize realized or potential safety concerns and engage in reporting when appropriate
c. Speak up when concerned about a potential safety concern
d. Demonstrate appropriate handwashing and universal precautions

7. **Interprofessional Collaborative Practice** –participate as a contributing and integrated member of an interprofessional healthcare team to provide safe and effective care for patients and populations. Students are expected to collaborate as a member of an interprofessional team, including:

a. Establish and maintain a climate of mutual respect, dignity, integrity and trust
b. Use attentive listening skills when communicating with team members
c. Help team members in need
d. Prioritize team needs over personal needs in order to optimize delivery of care
e. Demonstrate closed-loop communication

8. **Personal and Professional Development** –demonstrate the qualities and commitment required to sustain lifelong learning, personal and professional growth. Students are expected to:

a. Form clinical questions and retrieve evidence to advance patient care
b. Engage in self-reflection, self-appraisal, and self-directed learning
c. Demonstrate openness to feedback

Clerkship Specific Goals and Objectives

Upon completion of Surgery the student should be able to:

Educational Program Objective Domain	EPO Subdomains	Clerkship Learning Objective	Educational Events	Method of Assessment
● Knowledge for Practice (1)	● 1.2, 1.3, 1.5	**Knowledge for Practice –** demonstrate familiarity with common surgical diseases: relevant anatomy, disease process, diagnosis, treatment, and management	Faculty Lead Didactics	NBME Shelf exam
● Patient Care (3)	● 3.1, 3.2, 3.3	**History -** become competent at appropriately interviewing and examining patients with a surgical problem in at least one of a primary care, surgical clinic, or emergency department setting		Acuity evaluation
● Patient Care (3) ● Practice-Based Learning and Improvement (5)	● 3.2, 3.9 ● 5.6	**Physical Exam -** become competent at conducting focused examinations relevant to specific surgical settings		Acuity evaluation
● Patient Care (3)	● 3.6, 3.7, 3.8	**Patient Care-** develop an understanding of how to care safely for a potential or actual surgical patient		Acuity evaluation
● Patient Care (3)	● 3.1, 3.2, 3.3	**Patient Care -** become competent at appropriately interviewing and examining patients with a surgical problem in a primary care, surgical clinic, and emergency department setting		Acuity evaluation
● Patient Care (3)	● 3.3	**Patient Care -** enhance their clinical reasoning skills in applying medical knowledge to patient care		Acuity evaluation
● Interpersonal and Communication Skills (2) ● Patient Care (3)	● 2.2 ● 3.1, 3.2	**Patient Care -** participate or perform common basic procedures		Faculty / senior resident
● Patient Care (3)	● 3.6, 3.8, 3.9	**Patient Care -** assist in the care of the surgical patient as part of the patient care team		Acuity evaluation
● Professionalism (4)	● 4.2	**Practice Based Learning and Improvement**: students will refine their knowledge by attendance at conferences (e.g. QPI & Grand Rounds)		Conference attendance
● Interpersonal and Communication Skills (2)	● 2.1, 2.3	**Interpersonal and Communication Skills -** demonstrate interpersonal and communication skills that result in effective information exchange		Acuity evaluation

		between patients, their families, and professional associates		
● Interpersonal and Communication Skills ● Patient Care (3)	● 2.3, 2.4 ● 3.1, 3.2, 3.3	**Interpersonal and Communication Skills –** completion of history/physical, progress note and discharge summary with resident/faculty review		Documentation Card Received by Clerkship Coordinator
● Interprofessional Collaborative Practice (7)	● 7.1	**Systems Based Practice-** describe some aspects of the wider context in which health care is delivered	Faculty Lead Didactics	Conference attendance
● Interpersonal and Communication Skills (2) ● Patient Care (3) ● Professionalism (4) ● Practice-Based Learning and Improvement (5) ● Systems-Based Practice (6) ● Interprofessional Collaborative Practice (7) ● Personal and Professional Development (8)	● 2.3, 2.4 ● 3.9 ● 4.6, 4.7 ● 5.6, 5.8 ● 6.4 ● 7.1, 7.2, 7.3, 7.4 ● 8.2	**Professionalism –**conduct themselves in a manner befitting the role of 'physician' by maintaining a professional image in attire, grooming, and speech displaying compassion, empathy, and respect for all patients from diverse backgrounds attending to and honoring patients' privacy and confidentiality using constructive feedback to improve performance displaying honesty and integrity in all interactions, including admitting mistakes and sources of information timely completion of administrative tasks.		Acuity evaluation Records kept by the Clerkship Coordinator
● Personal and Professional Development (8)	● 8.2	Students will demonstrate an ability to self-appraise and an openness to accepting constructive feedback.		Mid-rotation feedback

Entrustable Professional Activities

1. EPA 1: Gather a history and perform a physical examination
2. EPA 2: Prioritize a differential diagnosis following a clinical encounter
3. EPA 3: Recommend and interpret common diagnostic and screening tests
4. EPA 4: Enter and discuss orders and prescriptions
5. EPA 5: Document a clinical encounter in the patient record
6. EPA 6: Provide an oral presentation of a clinical encounter
7. EPA 7: Form clinical questions and retrieve evidence to advance patient care
8. EPA 8: Give or receive a patient handover to transition care responsibility
9. EPA 9: Collaborate as a member of an interprofessional team
10. EPA 10: Recognize a patient requiring urgent or emergent care and initiate evaluation and management
11. EPA 11: Obtain informed consent for tests and/or procedures
12. EPA 12: Perform general procedures of a physician
13. EPA 13: Identify system failures and contribute to a culture of safety and improvement

Part 4: Clerkship Policies, Procedures, and Best Practices

Syllabus Change

Except for changes that substantially affect implementation of the evaluation (grading) statement, this syllabus is a guide for the course and is subject to change with advance notice.

Dress Code

It is important to follow the institutional dress policy. This may be different between institutions. It is your responsibility to make sure that you are aware of the policy at the institution where you are conducting your clerkships. In general, please remember:

- Patient care areas are "fragrance free," meaning no cologne, perfume, scented hairspray, strongly scented laundry, etc.
- Your white coats should always be accessible. There may be some locations of care where white coats are not worn.
- Just in case there is confusion, always have your white coat with you.
- It is advisable to change clothes before leaving the hospital after your work hours

Remediation

Remediation that adheres to current remediation policies and procedures must occur any time a student does not successfully pass a course or clerkship.

WFUSM's current remediation guidelines entail a remediation process that involves identification of deficiencies, development of a learning plan that may include measures (e.g., a test, a CPX, an OSCE) to ascertain that the deficiencies have been corrected, and the student has achieved the course/clerkship learning objectives. This plan is agreed upon by the student and the course or clerkship director and documented in a written, which must be approved by Associate Dean for that phase of the curriculum.

Duty Hours

We adhere to the following duty hour restrictions:

- A student may be scheduled for an average of 80 duty hours per week during a four-week period.
- Duty hours must be structured to allow a minimum of four 24-hour time periods free over a four-week time period for the duration of the clerkship.
- Duty hours may not exceed 24 continuous hours, following which students should have at least 14 hours duty free.
- Duty hours may include overnight call or shifts. The maximum frequency of overnight calls is once every third night. For students completing overnight shifts, the maximum number of overnight shifts is 5 nights sequentially. Following 5 overnight shifts, students will not be expected to return to duty hours until the next calendar day.
- Students must be excused from duty hours no later than midnight of the night prior to a shelf or final examination.
- Students are to be excused from duty hours on all days that have been identified as holidays on the WFUSM-MD program academic calendar.

Attendance and Participation

Immersion Year Policy: All absences must be submitted through the Student Scheduling and Absence System (SSAS) on Acuity when needing to be absent to an assigned location or service, whether you are sick or not; for the Clinical Operations subcommittee to be reviewed.

- **Planned Absences:** All SSASs for planned administrative time must be submitted **at least 21 days** before the beginning of the clerkship.
- **Unplanned Absences (sick or emergency):** Students must submit a SSAS within 24 hours if unable to do so prior to the absence.

Clerkship Specific Policy:
Winston-Salem Campus: Email Dr. Lauren McCormack and Emily Southern. The student is also responsible for alerting their team to the absence and anticipated return.

Charlotte Campus: Email Dr. Gaurav Sachdev and Shelly Williamson. The student must also email and haiku alert their team of their absence and anticipated return.

Clinical time, tasks, and assignments must be made up in accordance with the Satisfactory Academic Progression policy.

Didactic Attendance and Participation: *Wake Forest University School of Medicine expects students to participate in all instructional activities. Online didactics are no different from in person didactics in this regard.*

Clerkship Specific Policy on Artificial Intelligence: *Students may use AI tools on assignments in this course if permission is obtained from the course director in advance. This permission is given at the discretion of the course director, this permission may be revoked at any time, be limited to specific assignments, or be limited to specific AI tools. Any use without permission, or use outside of the previously defined allowable use, will be considered misconduct, and will result in a referral to the Honor Council.*

End of Course Evaluation

Engagement of students in the evaluation of our curriculum, and the collection of complete and actionable data for each offered course and clerkship, is an expectation of our accrediting bodies. In recognition of the importance of student course evaluation data in the effort to remain an accredited medical school, the Wake Forest University School of Medicine Undergraduate Medical Education Curriculum Committee (UMECC) has voted to require students to complete the end-of-course evaluation surveys for each course and clerkship. Consequently, survey completion within one week of course conclusion is a requirement for MD program participation. Students who do not fulfill this requirement will receive escalating support and attention.

Student end-of-course survey responses are highly confidential. Reports of student mistreatment on end-of-course surveys are handled in a highly confidential manner that necessarily involves student communication with the Director of Educational Compliance. **Only de-identified, collated data are provided to course directors (after the grades are issued), faculty, and governance committees for continuous quality improvement.**

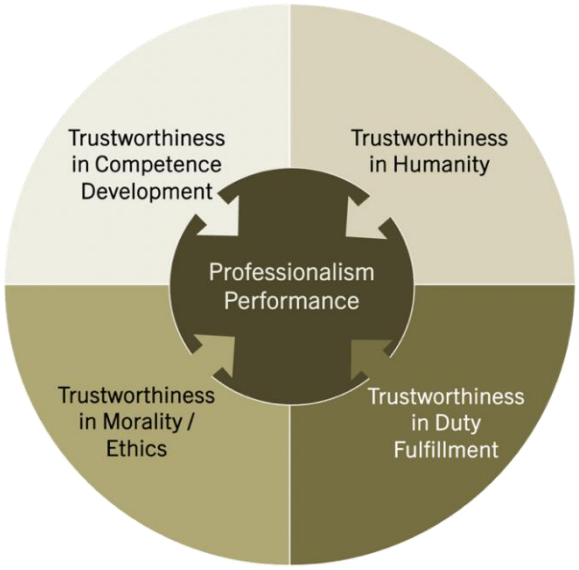
Student Behavior Policy/Professionalism

As part of their formal training, School of Medicine students learn the importance of demonstrating the attributes of a professional and understand the balance of privileges and obligations that the public and the profession expect of a health care provider. The importance of understanding and possessing the professional attributes expected of health care providers is emphasized throughout the school.

Students are expected to meet WFUSOM's standards of professionalism performance. Medical professionalism is defined as how our actions and behaviors demonstrate our commitments to the values, responsibilities, knowledge, and skills necessary for establishing our trustworthiness to serve as physicians. Professional trustworthiness consists of 4 domains, each of which apply to all of our professional relationships.

Summary:

- **Trustworthiness in competence**: The student fully engages with and monitors their learning to ensure they are developing the knowledge and skills needed to perform their professional duties, including incorporating feedback and seeking assistance when needed.
- **Trustworthiness in humanity**: The student demonstrates care and respect to ensure others' well-being, including patient-centered care, respectful teamwork, and safety practices.
- **Trustworthiness in morality/ethics**: The student acts ethically, including demonstrating honesty and transparency, owning their mistakes, and complying with the school's and medical center's policies and code of conduct.
- **Trustworthiness in duty fulfillment:** The student promptly follows through with all professional expectations and duties, including completing assigned tasks, responding to communications, maintaining learning resources and school spaces, and attending to their self-care and wellness needs.



The following table provides a more detailed description of our expectations of students' professionalism performance, organized by trust domain and virtue commitments:

Trustworthiness in COMPETENCE: Our demonstration that we have the requisite knowledge and skills to perform our professional duties		
Virtue commitment:	**Expectations of Students:**	**Themes:**
Commitment to excellence	Students will be fully **prepared** as instructed for all learning events. Students will invest appropriate **effort** into in-class, clinical workplace, and assigned outside-of-class learning activities. Students will **engage** in formative, self-regulated learning activities (e.g., practice exercises) to monitor and enhance their academic progress.	*Accountability for one's own learning through:* - *Preparation* - *Effort* - *Engagement*
Commitment to ongoing improvement (growth mindset)	Students will seek, be receptive to, and incorporate **feedback** from others on their performance. Students will **seek assistance** as needed and take steps to address their knowledge and skill deficiencies. Students will work to identify and mitigate their own **implicit and cognitive biases.**	*Self- improvement* *Feedback* *Help-seeking for learning needs* *Metacognition*
Commitment to intellectual humility	Students will **take ownership of limitations** in their knowledge, skills, and experience, seeking help appropriately for tasks beyond their current knowledge or skill level.	*Ownership of limitations* *Acknowledging*

	Students will acknowledge that complexity, ambiguity, and **clinical uncertainty** are inherent to the study and practice of medicine.	*innate uncertainty*

Trustworthiness in HUMANITY: Our demonstration that we care about and respect others and will do what is in their best interest to ensure their well-being		
Virtue commitment:	**Expectations of Students:**	**Themes:**
Commitment to respectful collaboration and a safe, inclusive work and learning environment	Students will **work cooperatively and respectfully with others**, demonstrating respect for others' values, beliefs, opinions, and perspectives, regardless of differing personal identities (e.g., race, religion, sexual orientation, gender identity, age, or other differentiating factor). Students will provide **feedback to peers** respectfully and effectively (i.e., specific, timely, and actionable). Students will **recognize others' concerns** and actively contribute to effective conflict resolution.	*Respectful teamwork* *Respectful, effective feedback* *Effective conflict resolution*
Commitment to own and others' safety	Students will ensure they meet all requirements according to the school's Medical **Health Requirements** and Immunization Policy. Students will demonstrate correct **hand hygiene** and use of **personal protective equipment** during lab and patient care activities. When sick with a potentially communicable illness, students will promptly contact Employee Health and follow their instructions regarding the need for testing and when they are permitted to return to in-person learning activities and clinical care of patients. Students will identify and report systems errors and safety threats/occurrences.	*Upholding institutional policies regarding health and safety*
Commitment to patient-centered care	Students will demonstrate **respect toward patients**, actively seeking to understand patients' values, beliefs, perspectives, and concerns. Students will explore new or unfamiliar beliefs and perspectives in preparation for future encounters. Students will demonstrate **empathy and compassion** toward patients. Students will use **inclusive language** when referring to patients verbally or in writing.	*Patient respect* *Active learning of patient identities* *Empathy / compassion*
Commitment to healthcare equity & social justice	Students will promote **justice, impartiality, and fairness** regarding access to resources and support for all patients. Students will acknowledge historical and current systemic contributors to medical mistrust, trauma, and inequities.	*Promoting fair and equitable access to care*

Trustworthiness in MORALITY/ETHICS: Our demonstration that we will act ethically and do the right thing in all of our work		
Virtue commitment:	**Expectations of Students:**	**Themes:**
Commitment to honesty and integrity	Students will be forthright, **truthful, and fully transparent**, including honesty regarding their own vs. others' contributions to group work, appropriate citation of information sources used, and in all patient	*Complete self / group honesty* *Disclosing COI*

	care documentation. Students will disclose all known conflicts of interest (COI) as per the school's COI Related to Student Assessment Policy. Students will fully adhere to the school's **Code of Honor and Professional Conduct** and exam administration policies. Students will **take responsibility for their mistakes** and proactively take steps to prevent future errors.	*Compliance with institutional policies and honor code* *Ownership of mistakes*
Commitment to core ethical principles	Students will appropriately apply the principles of autonomy, beneficence, non-maleficence, and justice to navigate issues in clinical practice.	*Application of ethical principles to clinical practice*
Commitment to patient confidentiality	Students will maintain **confidentiality of patient information** before, during, and after patient encounters.	*Confidentiality*
Commitment to appropriate interpersonal boundaries	Students will discern and maintain **appropriate interpersonal boundaries** with others in educational and work settings.	*Interpersonal boundaries*

Trustworthiness in DUTY FULFILLMENT: Our prompt follow-through on all of our professional expectations and duties		
Virtue commitment:	**Expectations of Students:**	**Themes:**
Commitment to professional presentation	Students will **dress professionally** for learning and/or clinical activities requiring it, according to the school's Dress Code for Medical Students Policy.	*Professional attire*
Respect for others' time	Students will **arrive on time** for all attendance-required educational/clinical activities and scheduled meetings. Students will **respond promptly to communications** from others (peers, staff, faculty, and leadership).	*Punctuality* *Prompt responses to communications*
Ownership of professional responsibilities	Students will submit assignments and **complete tasks promptly**, according to the assigned deadline. Students will ensure assignments are written clearly, accurately, and completely. Students will contribute an appropriate amount of **effort** to group work. Students will **submit absence requests promptly** and as instructed per the school's Attendance Policy. Students will communicate promptly with instructors/course directors regarding absences from required educational/clinical activities and make-up assignments as instructed per the school's Attendance Policy.	*Timely task completion* *Appropriate effort in group work* *Timely absence requests*
Commitment to maintaining educational spaces, tools, and resources	Students will ensure educational spaces (small group rooms, simulated exam rooms, lab spaces, sim rooms, clinical work rooms, student lounge areas, etc.) are left **clean and tidy**, with tools and furniture placed back in their original locations, following use.	*Maintaining shared spaces / resources*

Commitment to systems processes to improve curriculum quality	Students will complete the end-of-course/clerkship evaluation survey, providing **constructive feedback**, including specific and actionable next steps.*	*Constructive curriculum feedback*
Commitment to self- care	Students will actively foster their own **wellness** and **resilience**. Students will **seek assistance** for psychological and/or physical needs that may impact their academic performance or result in time away from the curriculum being necessary.	*Wellness activities* *Seeking resources as needed*

* WFUSOM requires that students complete end-of-course evaluation surveys for each course and clerkship within one week of course conclusion. Student responses on end-of-course surveys are confidential; only de-identified, aggregated data are provided to course directors, faculty, and governance committees for continuous quality improvement. Reports of student mistreatment on end-of-course surveys are handled in a highly confidential manner that necessarily involves student communication with the Director of Educational Compliance.

If students are unable to meet the above professionalism expectations, performance feedback will be provided to the student, and the student will be expected to take measures to address the concerns identified. Assistance from Student Affairs and/or the Center for Personal and Professional Development (CPPD) will be offered to students to help them to develop effective strategies to address these concerns.

Infractions of the Code of Honor and Professional Conduct will be managed according to the procedures as outlined in this section of the WFUSOM MD Program Student Handbook and per the Student Professionalism and Academic Review Committee (SPARC) Policy.

Appeal of Grades, Summative and Narrative Comments

Students that have questions or concerns about their final grade, summative or narrative comments should refer to the Appeal of Grades, Summative Assessments, and Narrative Comments policy. This policy can be found in the student handbook in PolicyTech.

The Family Educational Rights and Privacy Act of 1974 (FERPA)

FERPA prohibits release of student information related to academic performance without students' written consent. the following information concerning students is considered by the School to be directory information and may be disclosed without a student's consent, unless the student indicates in writing and submitted to the Registrar that any or all of the items are not to be released: Name; Address; Telephone number; Electronic mail address; Date and place of birth; Major field of study; Enrollment status; Dates of attendance; Degrees and/or awards received at the School; Participation in officially recognized activities; The most recent previous educational agency or institution attended by the student; and Other similar information, such as a photograph. If you have questions about FERPA please contact the Registrar's office.

Early Alert

As your director and coordinator, we are committed to supporting YOUR academic success in this course. For that reason, if you demonstrate any academic performance, professionalism, or behavioral problems which may impede your success, we may discuss and attempt to resolve the issue with you, or we may, if the situation requires, forward concerns to the appropriate person or group (ACA, CAWS, SPARC, etc.) to seek support and assistance in the matter. The goal is to make your learning experience in this course as meaningful and successful as possible. You are also encouraged to approach your director and coordinator if you are experiencing any challenges related to the class.

Student Outreach Services

Care Team: The CARE Team exists to ensure that all Wake Forest School of Medicine students have access to appropriate support as they adjust and succeed in demanding health sciences programs. This team provides a centralized system of prompt communication, coordination, and response for students in need of further support or at times of distress. Distress may include but is not limited to academic, financial or medical concerns, social pressures, personal well-being, or safety and security concerns.

You may contact the CARE Team via the online form, emailing the CAREteam@wakehealth.edu or calling campus/hospital security if the issue is an emergency.

Learning Environment/Mistreatment Reporting

The Liaison Committee on Medical Education (LCME) states, "Mistreatment, either intentional or unintentional, occurs when behavior shows disrespect for the dignity of others and unreasonably interferes with the learning process." Examples include discrimination and harassment; public humiliation; physical harm or threats of harm; and threats of lower evaluations or lower grades.

Wake Forest School of Medicine is committed to creating and maintaining a learning environment that supports and encourages respect for every individual and promotes the development of professionalism among medical students. If you experience or witness an instance of mistreatment or learning environment concern, we encourage you to report the issue by reaching out the Learning Environment Liaison.

The Learning Environment Liaison office is the central reporting resource for professionalism concerns for all School of Medicine students, as well as for visiting students, non-School of Medicine clinical students and non-affiliated student interns.

All School of Medicine learners may report mistreatment or learning environment concerns. To report a learning environment or mistreatment concern, or to report a positive learning environment experience, please contact them using one of the following methods:

- Via phone at 336-713-3352
- Via email at edcompliance@wakehealth.edu
- Via the Learning Environment online reporting form (requires login credentials)
- Concerns may also be reported through the Compliance Hotline at 844-587-0825 or at atriumhealth.ethicspoint.com

Academic Honesty & Integrity/Honor Code/Violations Process

As with all WFSM courses, the WFSM Code of Honor and Professional Conduct is in effect at all times.

ADA Statement

The WFUSM is committed to providing reasonable accommodations for all persons with disabilities as mandated by the provisions of the Americans with Disabilities Act. If you have a disability that may have some impact on your work in this class and for which you may require accommodations, please contact Student Services so that such accommodations can be considered. Students who have disabilities and need accommodations should visit the **Center for Learning, Access and Student Success** and should follow the guidelines provided in the **Student Disability Accommodations Requests policy and procedures.**

The Center for Learning, Access, and Student Success (CLASS) will assist and advise students with documented disabilities in arranging academic support and reasonable accommodations. Accommodations will be arranged on an individual basis, specific to the student's needs. The goal is to provide students with disabilities with necessary accommodations so they may compete on an equal basis in the classroom with their peers. Students must meet with a representative from CLASS and provide all necessary documentation prior to receiving accommodations. Absent significant extenuating circumstance (as determined by CLASS), accommodations will not be retroactive. There may be an expiration date on the accommodations provided and the same accommodations may not be available on STEP exams.

CLASS serves as a resource to students, faculty, and staff on issues related to compliance with the Americans with Disabilities Act of 1990, the Americans with Disabilities Amendment Act of 2008, and Section 504 of the Rehabilitation Act of 1973.

Campus Safety and Security

Security Services strives to provide a safe and secure environment, at all times, for our students, patients, visitors, and employees. Security Services operates 24 hours per day, seven days a week.

Emergency Preparedness

1. Program Security Services' telephone number into your cell phone
    a. Wake Forest School of Medicine Campus Security: **336-713-1568**
    b. Atrium Health Wake Forest Baptist Medical Center Security: **336-716-9111**
    c. Atrium Health Carolinas Medical Center Security: **704-355-3333**
2. In the event of an emergency, contact Security Services and/or dial 911.
3. If you "See Something, Say Something." Report to Security Services behaviors or suspicious packages that leave you feeling concerned, worried, or alarmed.
4. Sign up for the hospital/campus Emergency Notification System, which can be accessed at:
    a. Charlotte: https://peopleconnect.atriumhealth.org/departments/emergency-management/_resources/site-pages/emergency-notification-system
    b. Winston Salem: https://wakeready.wfu.edu/alert-methods/text-alerts/
5. Familiarize yourself with the various emergency situations and responses

If you have any questions regarding campus safety and security, contact Security Services.